

Removing the breast (mastectomy)

Surgery is one of the main treatments for breast cancer. Sometimes your surgeon may recommend having the whole breast removed (mastectomy).

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What is a mastectomy?

Breast surgeons will try to do an operation that means you can keep your breast ([breast-conserving surgery](#)). But sometimes they may recommend a mastectomy. This may be when:

- the lump is large compared to the rest of your breast
- there is cancer in different parts of the breast – this is called multicentric breast cancer
- there is widespread [DCIS](#) in the breast
- the cancer is on the skin or in underlying muscle
- you have had radiotherapy to the chest before to treat another cancer, such as a previous breast cancer or [Hodgkin lymphoma](#)
- you have a [family history of breast or ovarian cancer](#) and have tested positive for a [gene mutation](#).

Breast reconstruction

If you are having a mastectomy, your surgeon will usually ask if you want a new breast shape made at the same time. This is called an immediate [breast reconstruction](#). You can choose to delay breast reconstruction until after you have finished radiotherapy or chemotherapy. You may choose not to have reconstruction at all. You may decide to wear a false breast instead. This is called a prosthesis. We have more information about [breast prosthesis](#).

Breast reconstruction may not be suitable for everyone. This is usually if you have medical conditions that might increase the risk of complications during surgery and after.

You may decide not to have breast reconstruction at the same time as your mastectomy. If you think you might want breast reconstruction in the future, talk to your surgeon about this before you have a mastectomy. If you are having radiotherapy, some surgeons will recommend you delay breast construction until after you have had the radiotherapy.

Breast reconstruction is specialised surgery. It is usually done by a plastic surgeon or oncoplastic breast surgeon. They are experts in breast cancer surgery and reconstruction. There are different ways of doing breast reconstruction. A new breast shape can be made using:

- a silicone implant
- your own tissue taken from another part of your body, such as the back or tummy
- a combination of an implant and your own tissue.

You may be able to talk to a surgeon who specialises in breast cancer and plastic surgery before your operation. They can talk you through your options for reconstruction.

About our information

References



Below is a sample of the sources used in our breast cancer information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

ESMO. Early breast cancer clinical practice guidelines for diagnosis, treatment and follow-up. 2019, Vol 30, pp1192–1220. Available from: <https://www.esmo.org/guidelines/guidelines-by-topic/breast-cancer/early-breast-cancer> [accessed 2023].

National Institute for Health and Care Excellence (NICE). Early and locally advanced breast cancer: diagnosis and management. 2018. Updated 2023. Available from: <https://www.nice.org.uk/guidance/ng101> [accessed 2023].

Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by [Dr Rebecca Roylance](#), Consultant Medical Oncologist and [Professor Mike Dixon](#), Professor of Surgery and Consultant Breast Surgeon.

Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

Date reviewed

Reviewed: 01 October 2023 | Next review: 01 October 2026

ⓘ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

How we can help

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help